

April 1-14, 2020

COVID-19

Municipal Assessment

Rapid Municipal-Level Needs and Gap Assessment Report

Introduction

The Housing Recovery and Reconstruction Platform (HRRP) was established in December 2015 to take over necessary post-earthquake housing reconstruction coordination from the Shelter cluster. HRRP - currently in its fourth phase (HRRP 4) - is primarily funded by DFID and CRS until July 2021.

HRRP focuses on advocacy and communications, addressing gaps and duplications and improving coverage and quality of technical assistance through three dedicated pillars, including General Coordination, Information Management and Technical Coordination to support the national government (National Reconstruction Authority (NRA), CLPIU and GMALI), 32 districts and 282 municipalities as well as key stakeholders throughout the post-earthquake housing recovery and reconstruction.

To support the remaining reconstruction needs, HRRP 4 prioritizes urban and retrofitting reconstruction, most vulnerable reconstruction, support to local governments to take the lead in reconstruction process as well as institutionalization of earthquake response learning.

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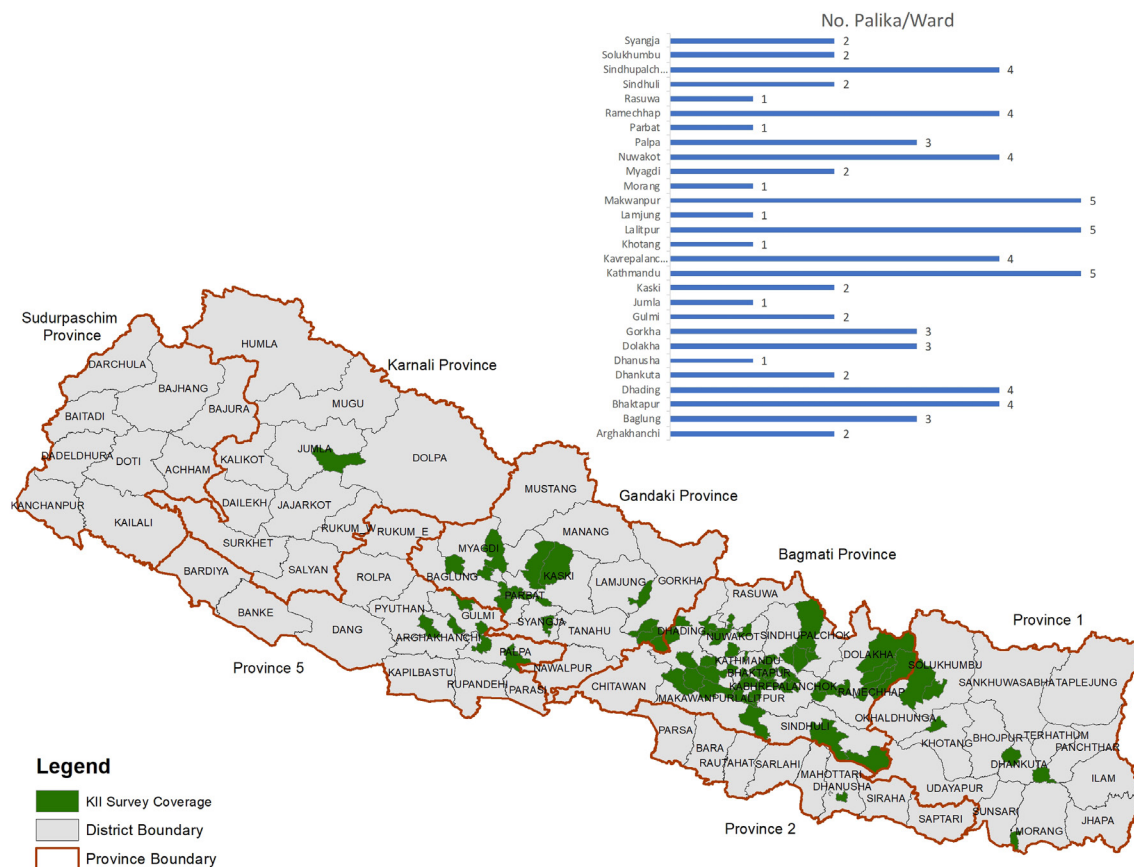
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Objective and Approach

This report will give a quick snapshot of general information on COVID-19 response activities, municipal level preparedness and planning, surficial needs and gap analysis and sense of coping mechanisms at local level.

The findings are to be shared among the responders and decision makers to help them be informed for proper planning and gap analysis.

Responses were collected over the phone from 71 municipalities, across 28 districts. [Questionnaire link](#). Considering the urgent need of information and data and human resource constraint only representative samples were collected and targeted mostly in earthquake affected districts.

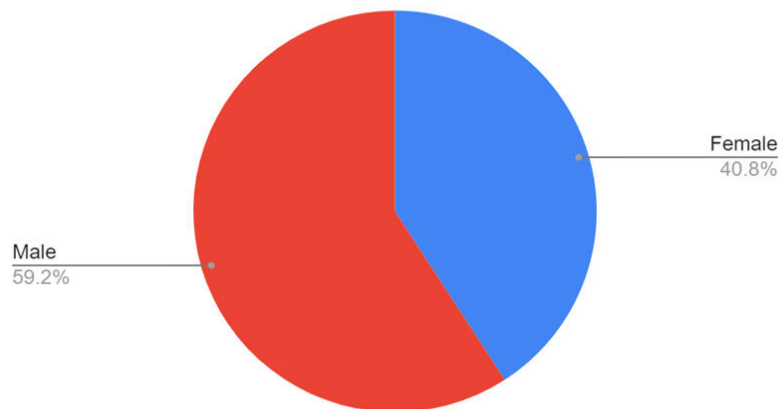


1. Demographic Information

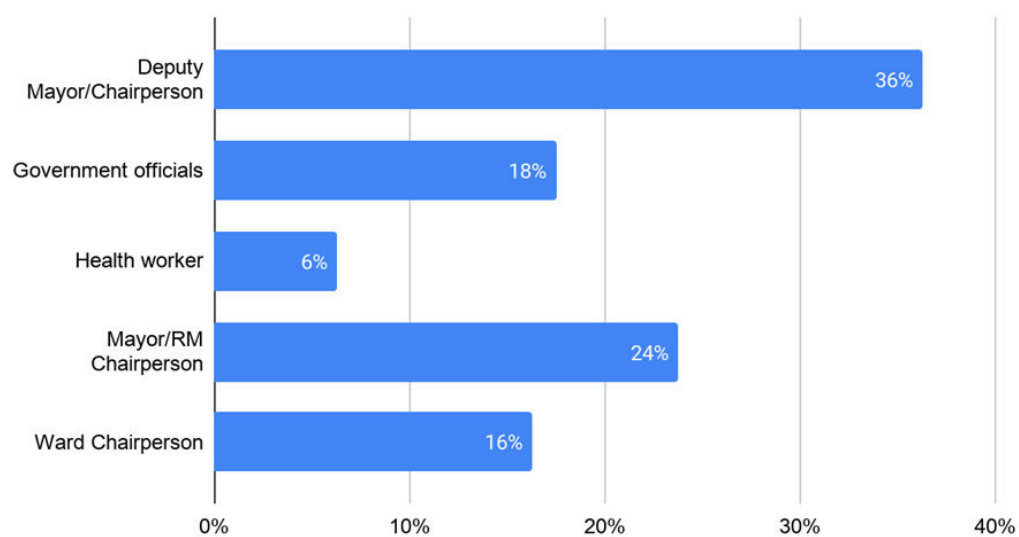
Of the **71 survey participants, 59% were male** and **41% were female**. A mix of respondents included Mayors, Deputy Mayors, Chairpersons, Deputy Chairperson,

Administrative Officers, Information Officers, Health workers and Ward Chief/ members.

Sex of respondent



Position of respondents



2. General Information on COVID-19

COVID-19 is the infectious disease caused by the most recently discovered coronavirus. As of April 21, 2020 there are 2.4 million confirmed cases and 163,097 deaths related to COVID-19, globally and 42 confirmed cases in Nepal (WHO; <https://covid19.who.int/>, NDRRMA; <https://covid19.ndrrma.gov.np/>)

The COVID-19 crisis affecting the world today requires a level of response that goes beyond the capacity of any country. The pandemic requires more attention, solidarity, preparedness and effective response in order to be eradicated and controlled.

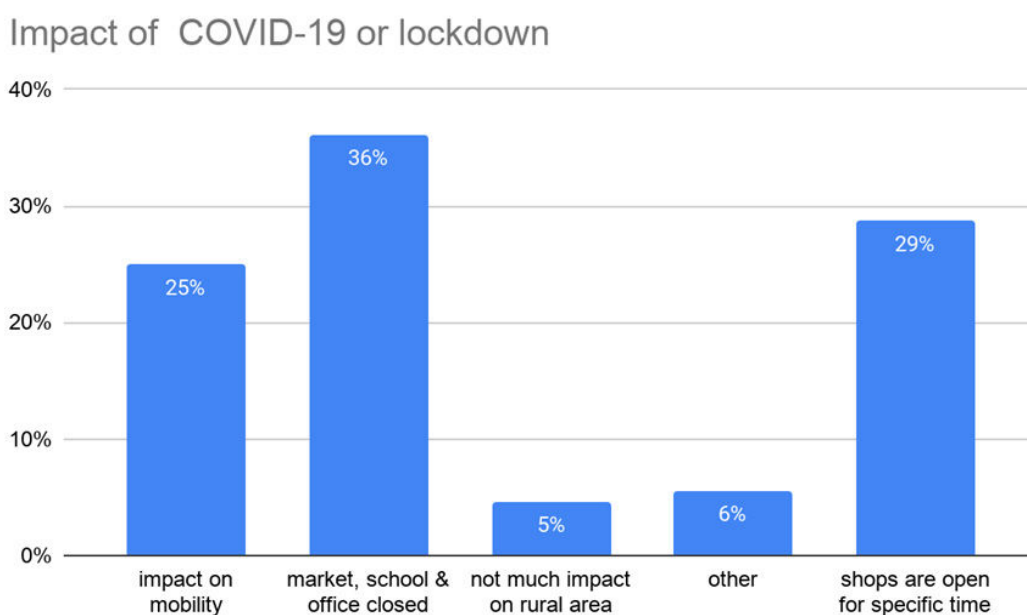
As the UN Secretary-General said: “More than ever before, we need solidarity, hope and the political will and cooperation to

see this crisis through together”. The Government of Nepal is putting in place a series of measures to address the situation, but more needs to be done, and international solidarity is required to ensure that the country is fully prepared to face the pandemic and address its impact in all sectors. Nepal is also vulnerable to natural disasters, including seasonal floods and landslides. There is a risk that the country could be faced with two emergencies at the same time. The Government of Nepal and the Humanitarian community which supports it, need to be prepared for this very real possibility (CERP, March 2020).

2.1 Impact on Livelihoods:

Since the 25th of March 2020 (the lockdown of Nepal as a precautionary measure to minimize COVID-19 cases) the greatest impact - reported by 36% of the respondents - has been seen in the functioning of service centers like markets, schools and offices. 25% of municipalities reported impact on mobility as a direct impact of the COVID-19, 5% reported

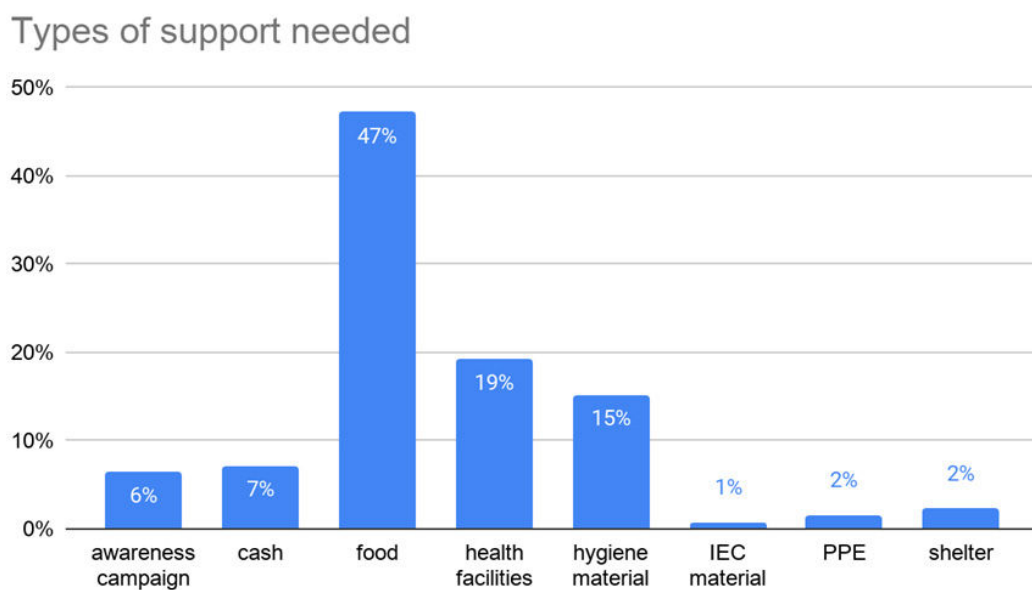
that the impact in mobility is less in rural areas. 29% of municipalities reported that shops continue to open for specific times providing access to essential items. Daily wage labourers, people with disability (PWDs) and elderly people have been most affected by the closure of markets and other services.



2.2 Overview of Support Needed:

The most sought-after assistance during the lockdown is food items and health and hygiene materials. 47% of the municipalities reported that they need food supplies to support families. Similarly, 19% of the

municipalities reported that they need support with the health facilities and materials in their community followed by hygiene materials.



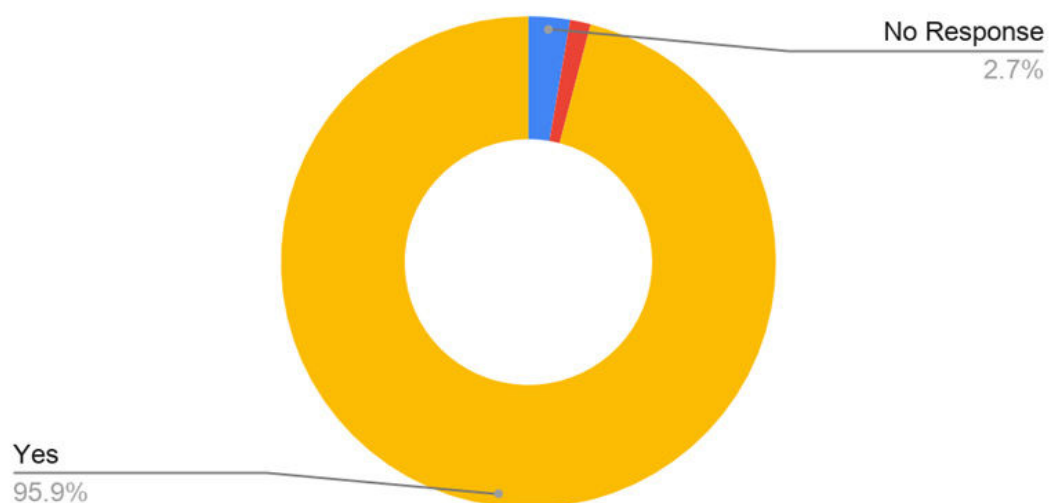
3. Health

3.1 Community Awareness:

96% of representatives reported that their communities are aware of COVID-19, its impact, and preventive measures to take. Almost all of the of wards/municipalities reported to have shared information on

COVID-19 to the general public, conducted awareness programs including door to door visits, radio broadcasting, and other mass media awareness campaigns in their communities.

Community awareness

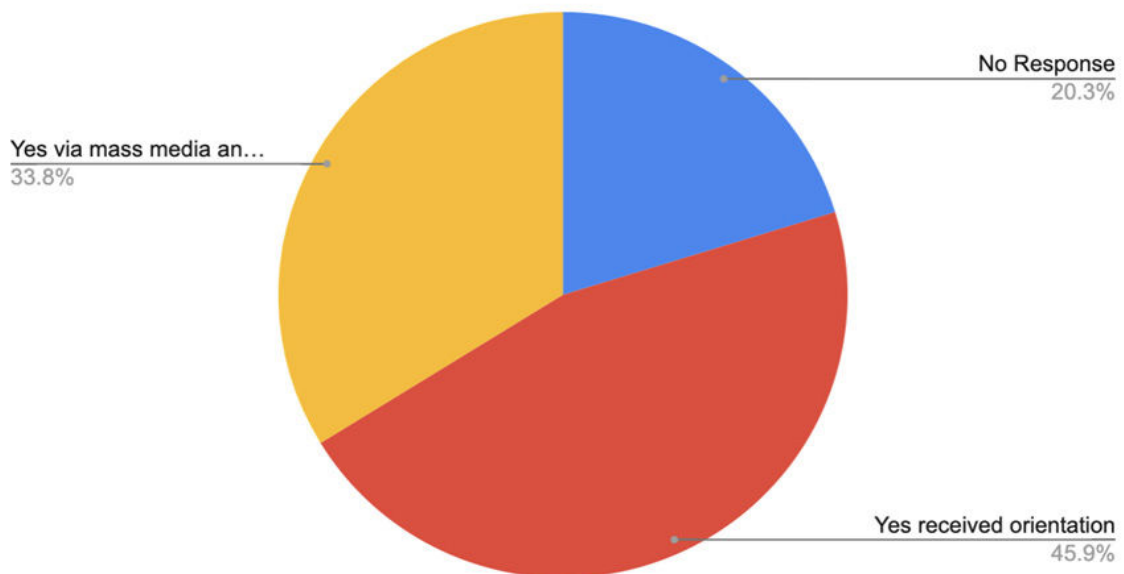


3.2 Health Worker Awareness:

All the health workers reported that they were aware of the COVID-19 situation in their respective area. **45.9% of health workers received orientation** from the province or ministry of health. 33.8% health

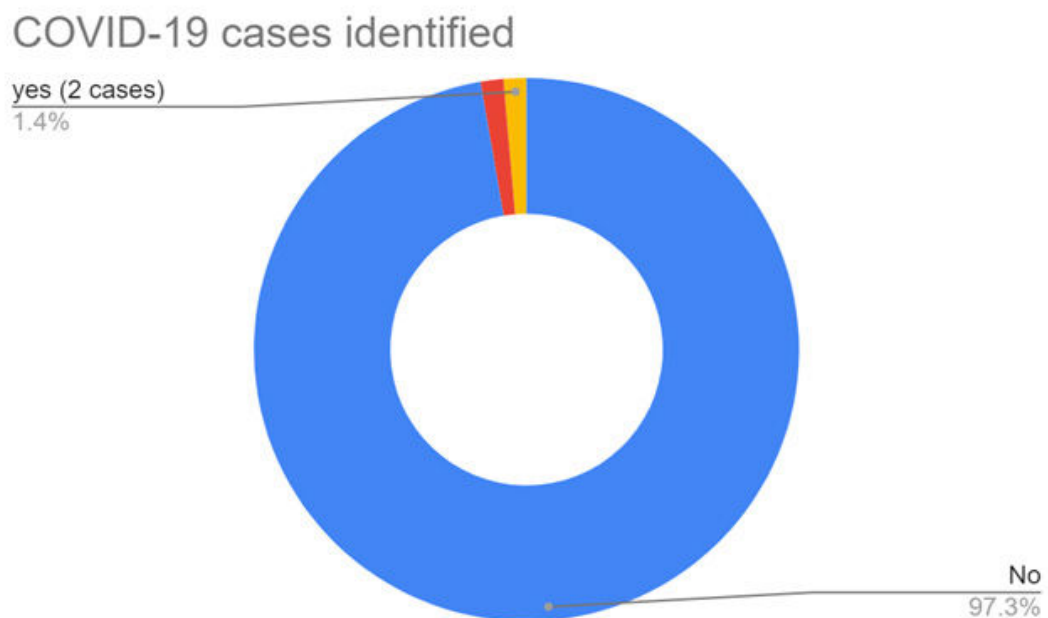
workers reported to have not received any formal orientations but were informed by local government or mass media to follow the guidelines provided by the government.

Health workers' awareness on COVID-19



3.3 Positive Cases Identified:

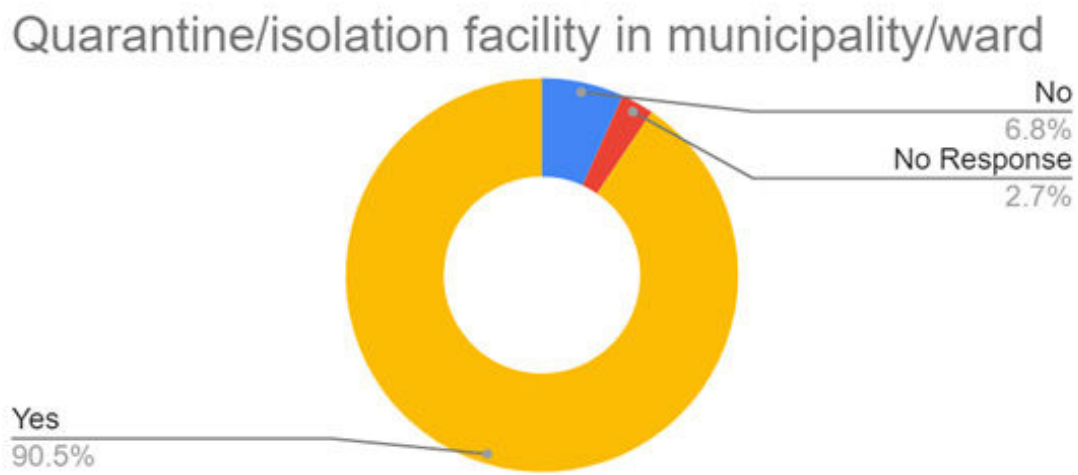
Only two of the 71 municipalities across 28 districts reported to have confirmed COVID-19 cases in their municipalities. Both positive cases were identified in the Baglung district.



3.4 Availability of Health Workers and Facilities:

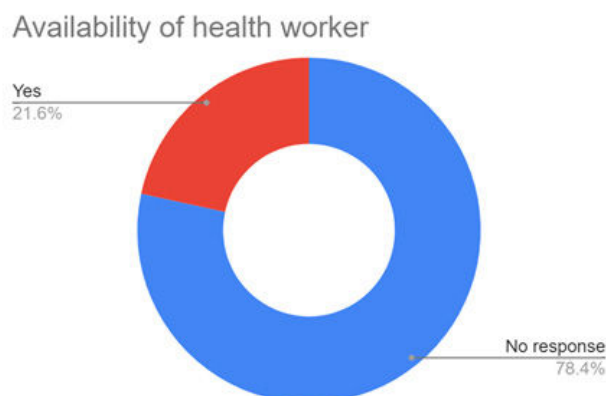
Among the municipalities involved in the survey, 90.5% have set up Quarantine and Isolation facilities, while 6.8% of the municipalities reported not having any

Quarantine and Isolation facilities, and 2.7% did not respond to the question.



Despite the large availability of quarantine and isolation facilities, only 21.6 % of municipalities reported having enough health workers in their municipality to use those facilities as per the standard operating guidelines others were not sure

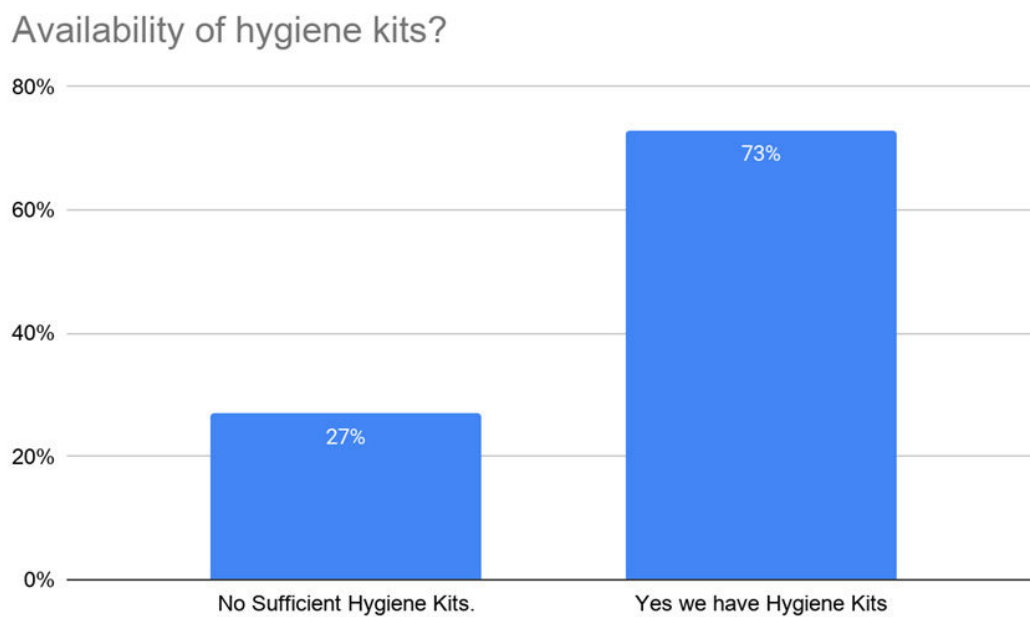
and did not respond. The adequacy of health facilities is also directly related to how much we can contain the spread of the disease. If the number of cases increases, extra health workers will be needed in all the areas.



3.5 Availability of Hygiene Supplies:

73% of municipalities reported that they have sufficient hygiene kits while 27% of the municipalities reported that they do not have sufficient hygiene kits available in their municipality. However, municipalities with sufficient hygiene kits expressed concerns

regarding the diminishing amount of kits with the extension of the lockdown. With market systems not functional as usual, there were serious concerns from the respondents over the procurement of additional supplies.



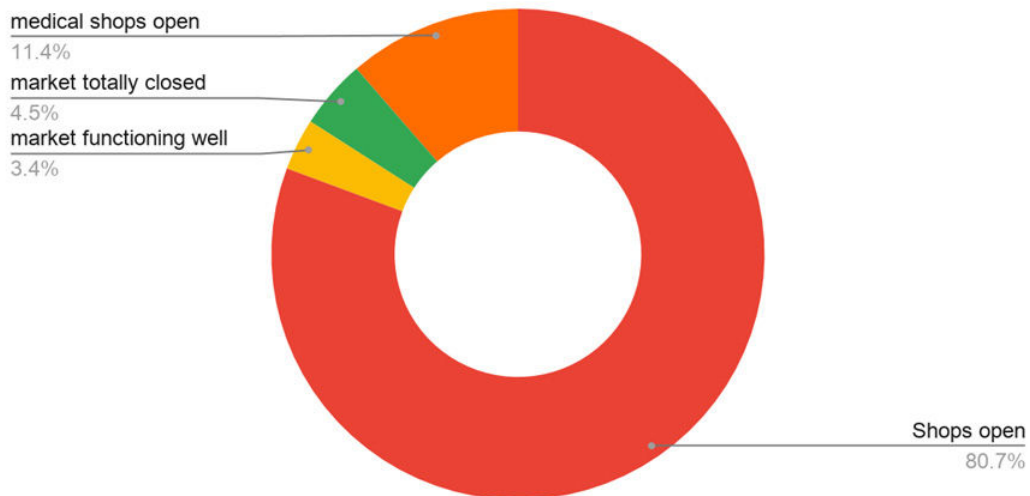
4. Food Security and WASH

4.1 Market Resilience:

80% of municipalities reported that the shops providing essential supplies like grocery and vegetable were still open in their areas. Only 3.4% of municipality reported that markets are functioning well

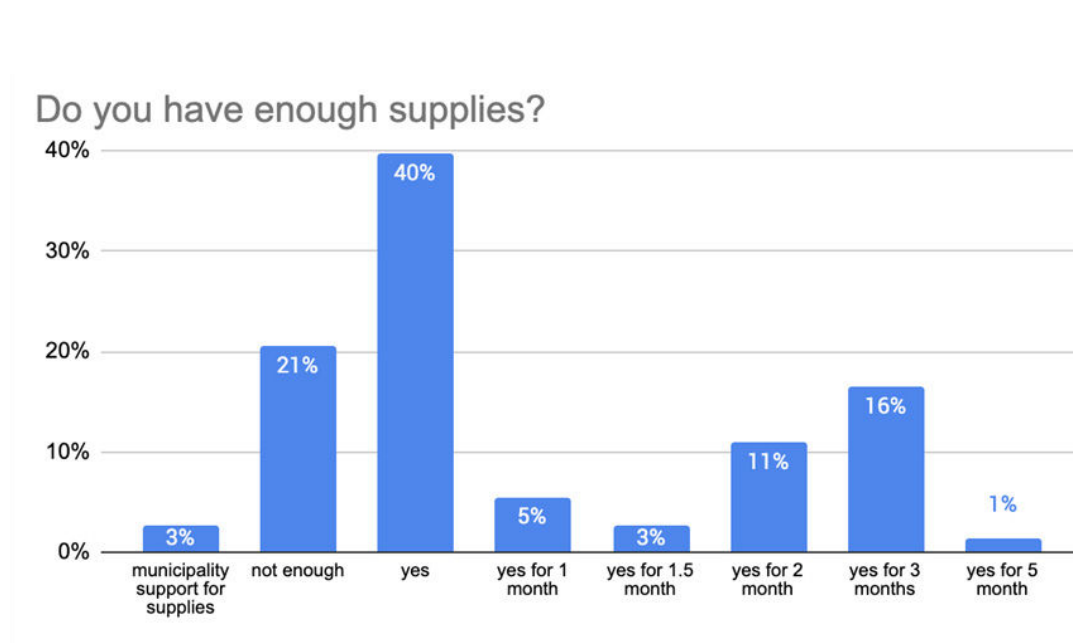
in the community area while in 4.5% of respondents reported that the markets are completely closed. Medical shops were open in only 11% of the municipalities.

How are the markets functioning (food and basic non food items)?



76% of municipalities reported that people in their municipalities have enough food supplies for 1-5 months whereas 21% municipalities reported that large population within the municipalities do

not have enough supplies. Only, 3% of municipalities reported that they were arranging the food/essential supplies from elsewhere.

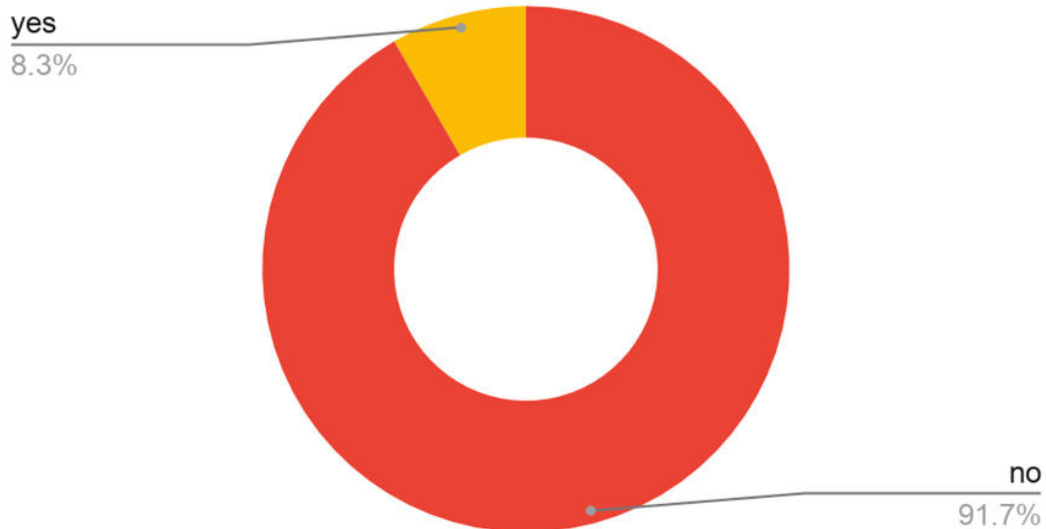


4.2 Water Challenges:

A vast majority of municipalities (91.7%) reported that water issues have not been experienced in their areas and reported having enough water supply as well as water sources being operational. However,

8.3% of municipalities reported that they are facing challenges in getting water as water plants are closed due to staff unable to work regularly.

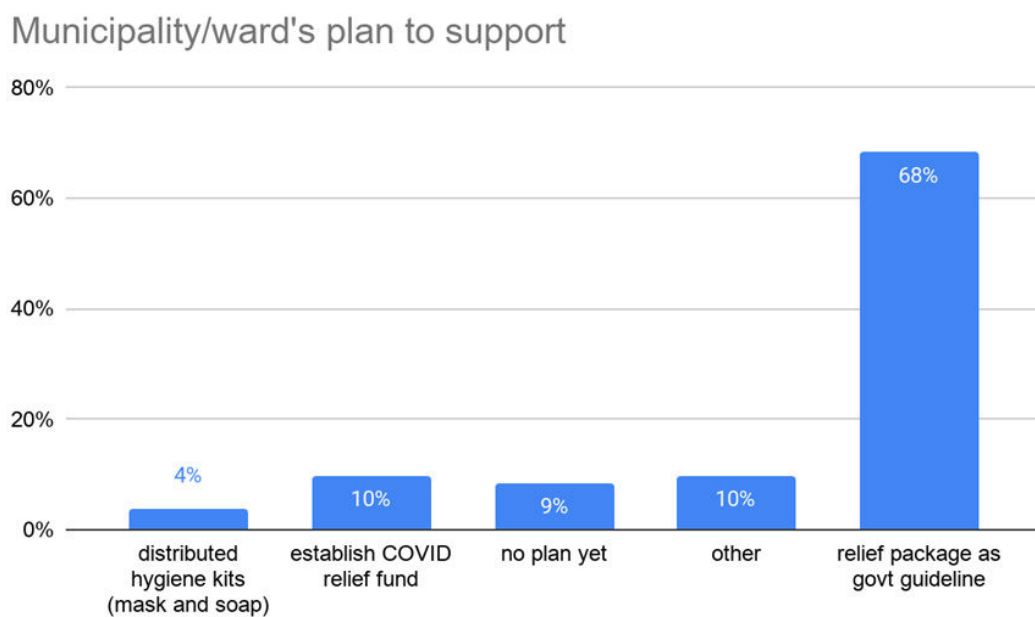
Do you see any water specific challenges amid



4.3 Municipality-Level Response Plan:

The majority of municipalities have developed response plans for COVID-19. 68% of municipalities have plans to distribute relief packages as per government guidelines, and 10% of

municipalities have established a fund to respond to COVID-19. Conversely, 9% municipalities did not have any plan during this data collection period.



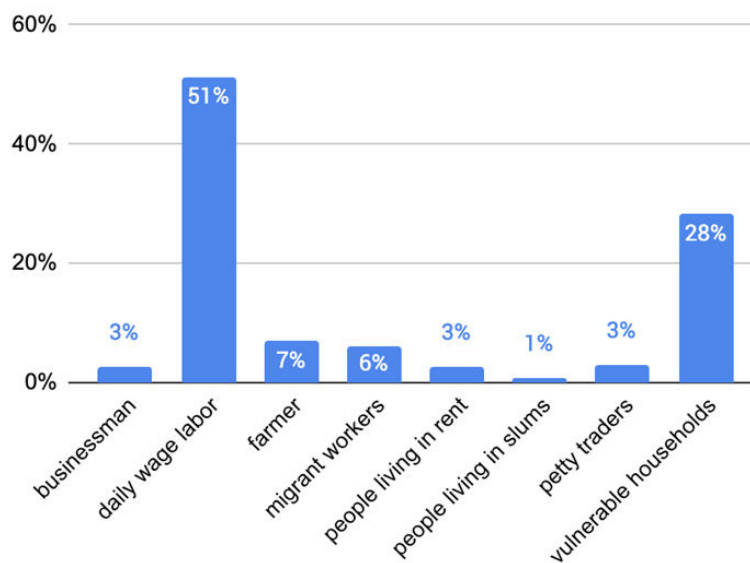
5. Protection and Vulnerability

5.1 Identification of Vulnerable Households:

Elderly people, daily wage laborers, farmers, migrant workers, and PWDs are most vulnerable due to the lockdown (51%). The second largest group of affected households were vulnerable households

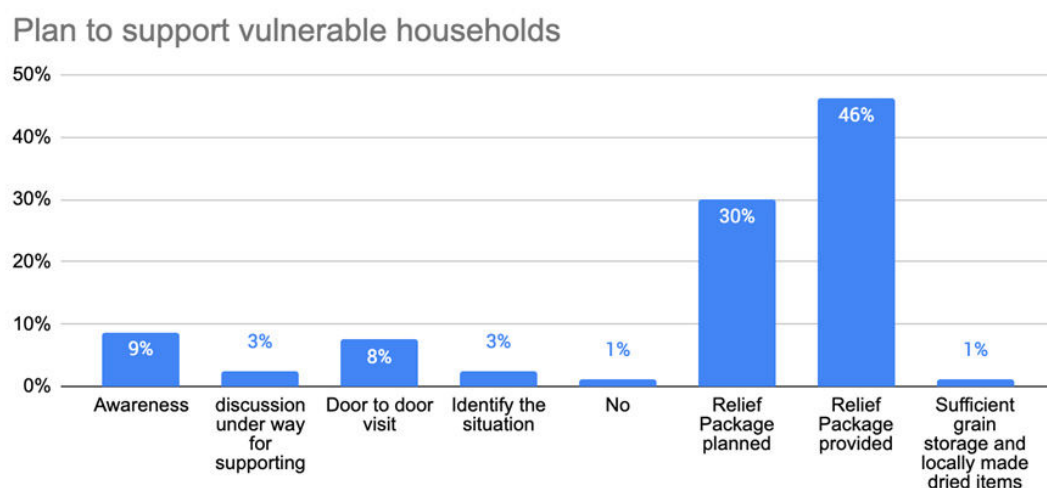
including single women, pregnant women, elderly, PWD, child headed households, landless households, marginalized communities (Chepang, Dalits), and economically poor households (28%).

Most affected/vulnerable due to COVID



5.2 Challenges and Solutions for Vulnerable Households:

The plan to support the vulnerable households is largely centred around providing them with relief packages. 46% of the municipalities reported to have already provided the relief and around 30% were planning to do so. Some additional support to the vulnerable include door to door visits, increased awareness about the vulnerable needs.



5.3 Migrant Workers:

All wards and municipalities reported that they are collecting information from individuals who recently arrived from outside the country and sending their swabs for analysis. During the data collection period, only 2 cases (Baglung) were found positive in surveyed districts and were linked to returnees from abroad.

Similarly, the majority of municipalities hired vehicles to provide transportation services to migrant workers to travel to their hometown and some wards/municipalities also provided food for them.

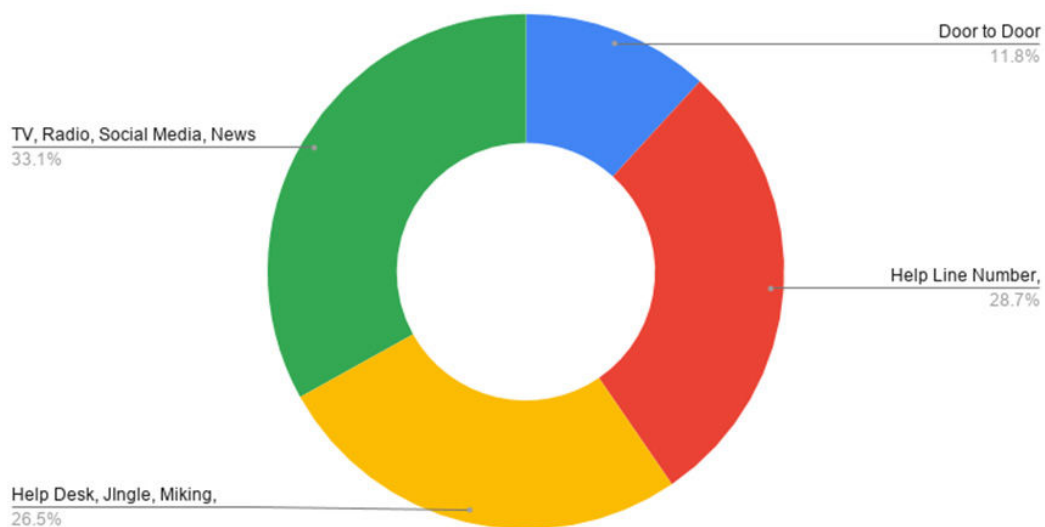
5.4 Discrimination:

Almost all municipalities reported that there is no discrimination against people in the wake of COVID-19 situation. However, social stigma against newly arrived people from India and other countries due to fears of coronavirus transmission were reported. The municipal representatives have been requesting people coming from abroad to stay in self quarantine and reported that the municipality has been supporting them as well.

6. Communication and Information Management

TV, radio and other social media are the most commonly used communication channels to disseminate information to the communities. The help desk help line numbers were also equally effective. Some municipalities also suggested door-to-door to be an effective communication medium.

What are the channels you use to share the information? Which are the top 3 channels people trusted most in your ward/municipality? Are community

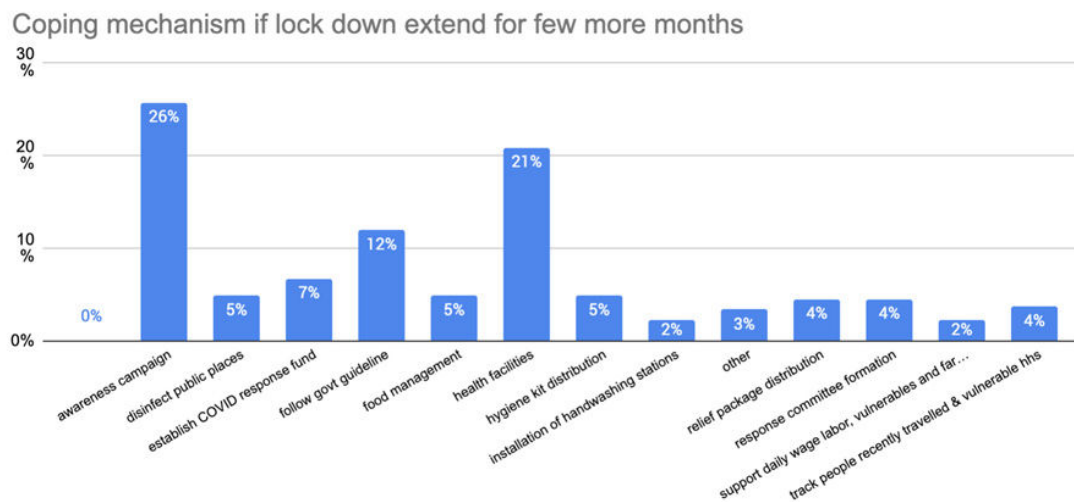


7. Current Coping Mechanisms

7.1 Ward/Municipality Strategies and Preventative Measures:

26% of the municipalities were conducting awareness campaigns on COVID-19 preventive measures, including stay home orders. Other measures taken include distributing masks, setting up temporary

quarantine and isolation facilities, door-to-door hand washing demonstrations, and distributing food supplies.



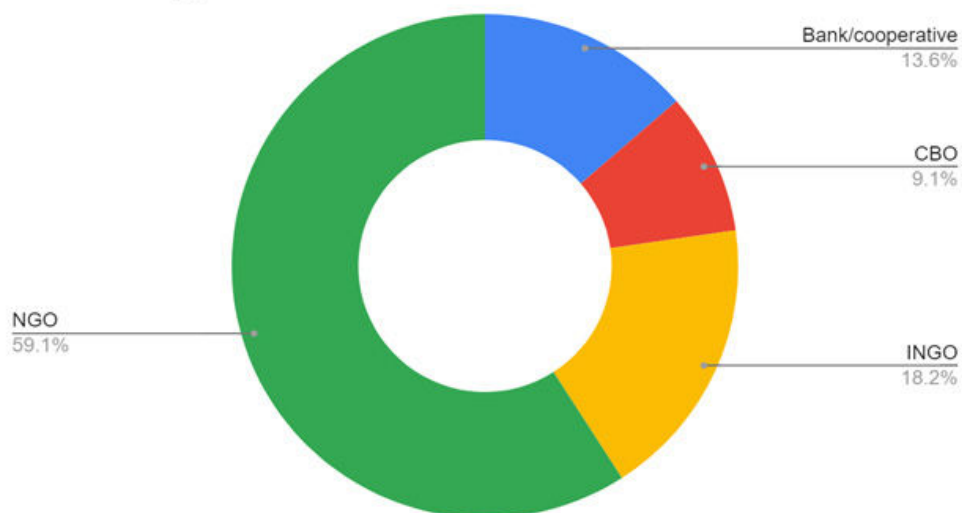
7.2 Outside Support:

All wards and municipalities reported having a plan to support their community members in the critical situation of lockdown. Besides local government, only 17 other organizations have supported or plan to support a few districts by providing hygiene kits, PPE, food packages and awareness campaigns. The organizations include, but are not limited to, Red Cross,

UN Habitat Nepal, Water Aid Nepal, and UNDP.

Out of 71 surveyed wards/municipalities only 31% have received outside support to respond to the COVID-19.

Outside support



8. Conclusion

- **Community awareness programs have been effectively implemented** in all municipalities. Mass media and social media are the most used communication channels to disseminate information to the communities.
- Most of the health workers are aware of the COVID-19 situation through the orientation and information shared by government and local bodies, **however formal training about the various guidelines are required.**
- Only a **few municipalities have a sufficient number of Quarantine and Isolation facilities** along with health workers and hygiene kits. If the number of cases increase, more quarantine and isolation facilities along with health workers and hygiene kits will be required. **The minimum requirements guidelines to be followed for quality assurance** and to meet other associate needs and to minimize infection risk.
- **Less access to Market in the majority of the municipalities** and is difficult in this situation, but grocery and vegetable shops are open in most of the areas.
- The government has provided a special package of response plans to the most vulnerable people due to COVID-19 (food etc.). Municipalities are providing these relief packages to the vulnerable families in their municipality as per government guidelines. The government has made a **one door system for the relief package and other support** that will be provided to the affected households.
- All the municipalities are collecting information of individuals who have recently arrived from outside the country and are **supporting the quarantine process and facilitating testing.** Most of the **municipalities have hired a vehicle to provide transportation for migrant workers** to their hometowns and some wards/municipalities have provided food for them. **COVID-19 related Information to be managed and updated regularly** two ways between central and local level.



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